ALI 5011

C Conference with [redacted] Ph.D, M.D, cleared for Secret contract.

C As an acquaintance of [redacted] in [redacted] (where he studied and lectured), I made an appointment to meet him at the American Psychiatric Association's ~~Convention~~ registration desk at their Convention headquarters, St. Louis, Mo, ~~for 6 May, 1954~~

B At an open house held for A.P.A. members interested in electro-encephalography, I had visited his laboratories at the [redacted] which seemed too insecure for conference. This was held, 12 to 12:40 P.M, 6 May, 1954 on the then unused balcony of the lobby of the auditorium-theater in use for convention meetings: Security briefing, ~~to~~ identify ~~you~~ the Agency inquiring.

C [redacted] appeared to be understanding of security, interested in the questions, and cooperative. He has conducted EEG and interrupted-light research for the [redacted] and is working on similar, present and

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future contracts; all of which are Unclassified. He offered to send reprints to my residence address, five of which have now come and been added to the office file on EEG.

He ~~knows~~ knew no present means of producing confusion, a convulsion with unconsciousness, in every subject by means of interrupted light alone (it is well known that some subjects are that sensitive). He has experimentally endured various intensities and frequencies for three hours continuously, without ill effects.

He believed that any subject could be convulsed by flicker, provided he were first sensitized by metrazol or an equivalent given intravenously. Different frequencies ~~may~~ have to be tried, running through a range of say 13 to 18 flashes per second; intensity, about 100 foot-candles. Most subjects seem most sensitive to 16 f.p.s.; a few; to 9. Forty seconds' exposure often suffice; a few minutes at most.

CT ~~He~~ Instead of metrazol, he routinely uses Azozol (Boots Pharmaceutical, New York), which is stronger. By mouth, Dromazol has been used by

CB [redacted], at the [redacted]. This is less positive a sensitizer; larger doses — to sensitize a higher percentage of subjects — are nauseating. Not known whether this or other nauseating sensitizing drugs could be safely combined with anti-emetic drugs, so as to ~~be~~ make it capable of sensitizing more subjects.

He did not know whether enough of a solar like amphetamine, or other drug, could be given by inhalation, to sensitize a majority of subjects to flicker-convulsion (or confusion).

If the subject were to be surrounded by a wall of a plastic substance like lucite a strong enough light behind this would be effective through it regardless of the subjects direction of gaze (closed eyelids do not protect).

He had no experience of any anti-convulsive drug peculiarly prophylactic against photogenic seizures. Numerous well-known anti-convulsants could be tried.

He thought that interrupted sound, being less effective than interrupted light, was hardly worth a trial. Combining sound with light seemed to him to add more complication than it offered gain — either toward making a subject more sensitive, or making more subjects sensitive.

Smell, continuous or interrupted, seemed to have been far less experimented with; such experiments as the literature offered had been unpromising, in his opinion.

C [redacted] interest has seemed sufficient, equally with his status as an EEG authority, for me to offer the suggestion that the Agency consider his use as a consultant.

A [redacted]